

Year 2023

- **September 2023: Initial Seizure & Symptoms**
 - **Event:** First focal seizure episode.
 - **Frequency:** 1/2 times a month.
 - **Symptoms (Lasting 1-2 min):** Aura of burning smell, urge of vomiting (no actual vomiting), lip smacking, and rushing to the washroom to prevent vomiting.
 - **Action:** Family consulted a gastroenterologist, ENT specialist, and pediatrician; problem remained unidentified.
 - **November 2023: Seizure Escalation & New Symptoms**
 - **Frequency:** Increased to once a week.
 - **Symptoms:** Started with burning smell aura, urge of vomiting, plus new symptoms: **left side lip twitching** and **broadening eyes** during the seizure.
 - **Action:** Continued consulting pediatrics.
 - **Later November 2023: Symptom Progression**
 - **Frequency:** Increased to 1-2 times a week (seizure lasting 1-2 minutes).
 - **Symptoms:** Burning smell aura, urge of vomiting, plus new symptoms: **not able to talk** (blabbering), and **nausea** for a couple of minutes after the seizure.
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Year 2024

- **January 2024: Diagnosis & Imaging**
 - **Action:** Consulting with a **Neurologist** began.
 - **Finding:** MRI suggested a **lesion in the midbrain-temporal lobe**.
 - **Action:** Neurosurgeons had varying opinions due to the complex location and mostly suggested **wait-watch-compare**.
 - **Medication:** Started **Levetiracetam**, which *did not* help control seizures.
- **February 2024: Visual Impairment Detected**

- **Action:** Neuro-ophthalmology evaluation performed.
- **Findings:** Vision and fundus were normal. **Perimetry/Visual field was impacted on the right side**, and **right side homonymous hemianopia** was detected.
- **April 2024: Lesion Check & Management**
 - **Finding:** Differential MRI (compared to Jan 2024) suggested the lesion was **static**.
 - **Status:** Seizures were still happening.
 - **Action:** Neurosurgeons suggested biopsy, direct chemotherapy, or continue wait and watch. Family decided for wait and watch.
- **July 2024: Continued Observation**
 - **Finding:** Differential MRI showed **no significant difference in the lesion**.
 - **Action:** Continued wait and watch.
- **August 2024: Seizure Control Achieved**
 - **Medication:** Seizures were finally controlled and reduced to **zero** with the following doses:
 - **Clobazam** 5 mg once a day | (0-0-5)mg.
 - **Carbamazepine** 200 mg twice a day | (100-100-200)mg.
- **September 2024: Worsening Ocular Issues**
 - **Action:** Neuro-ophthalmology re-evaluation done.
 - **Findings:** Right side visual field still impacted, eyesight vision degraded (major impact on **left eye**), and the left eye was intermittently squinting (toward the outside/further left).
 - **Action:** Started **spectacles (eye glasses)**.
 - **Imaging:** Repeated MRI showed no significant difference in the lesion. Continue to wait and watch for tumor.
- **December 2024: Motor Skill Decline**
 - **Symptom:** Child started to keep her **right hand fist usually closed** and experienced **degradation in handwriting**.

- **Imaging:** Repeated MRI showed no significant difference in the lesion. Continue to wait and watch for tumor.
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Year 2025

- **March 2025: Hand Use & Auras**

- **Symptom:** Less use of the right hand and occurrence of **seizure auras** (but not actual seizures).

- **April 2025: Surgery**

- **Action:** Child underwent **left pterional craniotomy with microsurgical excision of a mesial temporal space-occupying lesion (SOL)**. * **Result:** Only about **20% of the lesion** could be removed; the remaining 80% was left due to the complex location near the optical nerves.
- **Post-Op Status:** little relief in the left eye intermittent squint (potentially due to pressure relief, but still intermittent). The right hand was still less used, with the fist mostly closed. Vision was still affected (eyesight degradation and right side field vision).

- **July 2025: Pathology/Oncology Results**

- **Action:** Oncology reports came from two different labs for two parts of the tissue sample.
- **Findings (Negative):** Nothing found in IHC, FISH, or MGMT. Advanced NGS panel report from TATA was negative.
- **Finding (Positive):** Advanced NGS panel report from Medgenome showed **DBN1/BRAF fusion**.

- **August 2025: Treatment Planning**

- **Action:** Visited couple Oncologists.
- **Outcome:** Opinions are varying: some suggesting **targeted therapy** (Tovorafenib - BRAF or Trametinib - MEK) and others suggesting **chemotherapy (V+C) / Vinblastine**.
- **Next Action Plan :** Decide between Targeted v/s chemotherapy (**V+C**)/ **vinblastine**. Currently undergoing physiotherapy for hand and fingers.

- **October 2025: Enrolled to Tovorafenib Phase 2 Trial**
- **November 2025 to Feb 2026 :**
 - As Existing epilepsy drugs were not compatible with chemo and also less effective for trial drug , thus tappers and switched to following :
 - Lacosamide 100 mg – twice a day
 - Brivaracetam 15 mg – twice a day
 - Clobazam 1.25 mg – Night
- **March 20th – April 20th 2026 :**
 - Took 350 mg tovorafenib once a week for 4 weeks.
 - Started developing neurological symptoms like:
 - Right hand and grip weaker
 - Right eye vision drastically reduced.
 - Throat ulcer
 - Sodium low and drowsiness.
 - Appetite dropped to 25% of original intake.
 - Weight loss to 20 kg from 22 kg.
 - **April 20th to May 20th 2026 :**
 - Headache non bearable and not stopping despite taking ibuprofen / Paracetamol.
 - Sodium dropped to 125.
 - **24th April** Admitted to hospital for osmosis and high ICP.
 - **24th April Evening** Elective ventilator.
 - **25th April :** obstructive hydrocephalous detected in CT.
 - Episode of Neuropraxia / Stroke.
 - **27th April : VP shunt** Placed.
 - **28th April : Extubated ,** but right side weak and unable to speak.
 - **29th April : Craniotomy** happened.

- **1st May : Extubated.**
- **15th May : Discharged**
 - Still not wake up , not fully conscious.
 - Eyes not opening , hand lag week , Right side Hemiparesis.
 - No speech , no swallow.
 - Feeding via NG tube.
 - Started physio therapy at home and sodium management and NG Tube feed.
- **May 20th to June 20th 2026 :**
 - Conscious, Eyes opened.
 - Memory retained , Started discussions , mostly reactive.
 - NG tube removed , started eating by mouth.
 - Started walking.
 - Weakness and less strength in overall body.
 - Right hand and leg still not fully recovered , figure granular movements etc not achieved on right side.
 - Right side face muscle still not fully recovered.
 - Electrolytes stable.
- **June 21th 2026 till Date:**
 - Various Craniotomy Tissue based reports are now available.
 - Seeking further treatment advise.
 - Physio therapy / Occupational Therapy in progress.
 - Speech ,Facial, and Physical Recovery in progress.

